NCCP 00319 · clinician draft

Carboplatin + Oral Etoposide

Which treatment this guide covers This guide covers carboplatin plus oral etoposide for extensive-disease small-cell lung cancer in people unsuitable for cisplatin-based treatment. It does not cover an intravenous etoposide schedule or an added immunotherapy medicine. If the prescription differs, ask for its matching guide and consent discussion.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 2–3 · oral etoposide, then two-day post-treatment support

Active medicines: etoposide

Emetogenic risk: minimal to low

Medicine	Dose / route	When
Continue the reconciled Day1 prevention plan	As individually prescribed	Includes the Day2–3aprepitant and Day2–4dexamethasone from the carboplatin reference row where prescribed Do not repeat Day1doses or add a second high-risk course.
Oral-etoposide-day prevention / rescue	Individualised to daily risk, symptoms and existing medicines	Before oral treatment on Days2–3 and appropriate support for two days after finalSACT (Days4–5) NCCP00319 calls oral etoposide minimal–low. NCISV8p5provides a daily-risk principle, not an explicit universal Day5drug/dose; pharmacy must reconcile it.

Additional phase-specific support

- Prescribe one clear multi-day schedule with stop days.
- No automatic routine olanzapine, prolonged aprepitant, or copied five-day cisplatin schedule.
- Patient records exact oral doses and contacts team about missed/vomited doses.
- Regular premedication is not usually required unless previous hypersensitivity; no additional routine supportive medicines specified by00319.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00319 v5 · pNCCP00319pp1,4–5;NCISV8pp4–5 · NCCP NCIS Medical Oncology V8: multi-day principle

Sources checked 2026-10-26 · SACTCheck v0.84.23 · Regimen-level drafts; prescriber and oncology-pharmacy verification required